DIABETES IN LABOR: INSULIN AND IV FLUIDS

CHRISTUS St. Michael / OBHG · Texarkana

Last reviewed: Nov 2026

Companion to Diabetes in pregnancy · CSM OB insulin infusion protocol (CHRISTUS Health) · ACOG PB 190/201 · ADA Standards 2026 §6, §15–16 · Labor and delivery · Decision support only, not unit policy

HOW TO READ THIS CHART: Step 1 on admission. Each FSBG: find the row, set the insulin rate AND the fluid. **Below 70 = low: rescue box.** Hexagons are questions; shaded rectangles are actions.

SHADING KEY: WHITE = monitor / set-up · LIGHT GRAY = admission, no-drip rule · MID GRAY = insulin running · DARK GRAY = hypoglycemia, DKA

STEP 0: DIABETES IN LABOR

Diabetes (GDM A1/A2, type 2, type 1) admitted for labor, induction or scheduled cesarean. Unsure of type AND on home insulin, insulin pump, or prior DKA: treat as type 1 for basal (never a basal gap) and get senior/endocrine review now. Not on any insulin and no DKA history: diet/oral rule (no drip).

STEP 1: ON ADMISSION (CSM) · LIGHT GRAY

Stop all insulin, including SC insulin pumps (unless a signed CSM exception), and oral agents. NPO except 8 oz ice chips every 4 hours. POC glucose; BMP and urine ketones if FSBG >160. Type 1: start the IV floor before stopping SC/pump, or keep the glargine/detemir already given; never a gap. Night-before/morning home doses and steroid adjustment: per the CSM provider/pharmacy order set (this app gives no home doses).

DRIP SET-UP · WHITE

Regular insulin (Myxredlin) 100 units/100 mL, so units/h = mL/h; range 0.5–5 units/h. PE-lined tubing: fill with insulin, wait 20 minutes, then flush. Smart pump, dedicated line, independent double-check. High-alert medication. The CSM protocol runs on a provider order.

MONITORING · WHITE

FSBG before starting IV fluids.
Insulin drip running: FSBG every 1 hour, regardless of dilation (Michael override of CSM q2h latent; ACOG PB 201).
Insulin-requiring, drip not yet running: every 2 hours in latent labor or NPO before delivery; every 1 hour at 4–6 cm or with regular contractions (CSM).
Diet-controlled GDM or oral agents (no drip): every 4 hours until active labor, then every 1–2 hours (Michael decision 11 Nov 2026).
After any hypoglycemia rescue: recheck in 15 minutes (go-live resolution 2).
Hospital point-of-care glucose for insulin decisions; CGM for trends/alarms only.

TYPE 1: NEVER ZERO BASAL · MID GRAY

At 70–90 the floor is 0.5 units/h with D5LR 125 mL/h and provider notification; the provider may set 0.5–1 units/h by trend (prevents ketosis; not glucose correction). Clinician override of the CSM chart (CSM: hold), approved by Dr. Carozza 11 Nov 2026. 61–69: keep the floor AND give the hypoglycemia rescue; recheck in 15 min. The IV floor is not needed only if effective long-acting SC basal (glargine/detemir given within its duration) is on board, or a pump is continued under a signed CSM exception. Otherwise the pump stops only after the IV floor is running. Do not stack. Type 1 insulin stops only for FSBG ≤60 or symptoms, as an emergency pause: dextrose continues, rescue, recheck in 15 minutes, resume 0.5 units/h once >70 and rising (rising = higher than the previous check); a pause of about 1 hour or longer needs provider review.

STEP 2: FSBG NOW — WHICH ROW? (CSM NOMOGRAM, MAINTENANCE 125 mL/h)

CSM nomogram; maintenance fluid 125 mL/h. FSBG is read as a whole number in mg/dL. If a value has a decimal, round DOWN (60.5 → 60). 60 is the ≤60 row; 90 holds; 91 starts 0.5 units/h; 120 stays on D5LR; 121 switches to LR. Meter "LO": rescue now. Meter "HI": >250 row, call provider, DKA screen, lab glucose. No bolus. Maximum 5 units/h. Never use this chart to treat DKA.

BELOW 70: Any FSBG below 70, or symptoms at any value: hypoglycemia rescue now (rescue box). 61–90 means "no insulin increase," not "no action."

FSBG (mg/dL)	GDM A2 / type 2 on insulin	Type 1: insulin	Fluid at 125 mL/h
≤60 (CSM "<60" row; 60 included)	No insulin. Contact provider. Hypoglycemia rescue now; recheck 15 min.	Emergency pause (dextrose continues); resume 0.5 units/h once >70 and rising (rising = higher than the previous check).	**D5LR**
61–90 (insulin hold band; 61–69 = LOW, rescue)	Hold insulin (no added insulin). 61–69: hypoglycemia rescue now, recheck in 15 min, notify provider.	0.5 units/h floor (provider may set 0.5–1) + notify provider. 61–69: keep the floor AND give the hypoglycemia rescue; recheck in 15 min. Clinician override of the CSM chart (CSM: hold).	**D5LR**
91–120	**0.5 units/h**	0.5 units/h	**D5LR**
121–150	**1 units/h**	1 units/h; ketones at LR start, then q4h (Doc proposal)	**LR**
151–180	**2 units/h**	2 units/h; ketones at LR start, then q4h (Doc proposal)	**LR**
181–220	**3 units/h**	3 units/h; ketones at LR start, then q4h (Doc proposal)	**LR**
221–250	**4 units/h**	4 units/h; ketones at LR start, then q4h (Doc proposal)	**LR**
>250	**5 units/h (maximum; notify provider)**	5 units/h; ketones at LR start, then q4h (Doc proposal)	**LR**

DIET-CONTROLLED GDM (A1) OR ORAL AGENTS: NO DRIP · LIGHT GRAY

Diet-controlled GDM (A1) and anyone on oral agents (metformin, glyburide): NO insulin drip. Sliding scale insulin as needed, per provider order (no CSM scale in this app; no doses shown). FSBG every 4 h until active labor, then every 1–2 h. Call provider now for FSBG >180, ketones, or vomiting/illness. Low (<70): rescue box. The drip is for insulin-requiring patients: A2 on insulin, type 2 on insulin, type 1 (Michael decision 11 Nov 2026).

TYPE 1 ON PLAIN LR (≥121): KETONES · MID GRAY

Type 1 on plain LR: urine ketones (or beta-hydroxybutyrate) at the start of LR and every 4 h on LR (q4h = Doc proposal, not CSM), and now for nausea, vomiting, abdominal pain, fast breathing or 5 units/h with rising values. Any positive ketones: call provider; go to the DKA box; no ad hoc dextrose.

HYPOGLYCEMIA <70 OR SYMPTOMS: RESCUE NOW (CSM ADULT HYPOGLYCEMIC PROTOCOL) · DARK GRAY

D50 50 mL IV: FSBG <50 with IV access: alert and NPO or unable to swallow (NPO = unable to swallow), OR questionable consciousness (CSM; pharmacy).
D50 25 mL IV push: FSBG 50–69 with IV access: alert and NPO or unable to swallow, OR questionable consciousness (CSM; pharmacy).
Glutose 40% gel 30 g / 15 g buccal: ALERT, no IV, can swallow safely: 30 g if <50; 15 g if 50–59 (CSM). 15 g also at 60–69 (Michael-approved addition; pending CSM pharmacy build).
Glucagon (GlucaGen) 1 mg IM, ONCE: Only if ALERT AND no IV AND cannot swallow, FSBG <70; OR questionable consciousness, no IV, FSBG <60 (CSM). Also questionable consciousness, no IV, 60–69 (Michael-approved addition; pending CSM pharmacy build).
No IV in any rescue: Get IV access now (urgent provider / rapid-response call if consciousness is impaired).
Recheck FSBG in 15 minutes. Still <70: repeat D50 or gel per the matching row. Glucagon is ONCE only: if still <70 15 minutes after glucagon, get IV access for D50 and call the provider / rapid response. Notify the provider after every rescue.
Below 54 or any symptoms: rescue and call the provider immediately.
Symptoms but FSBG ≥70: repeat POC, send lab glucose, call provider. If you cannot check, treat as low.
D10 is not in the CSM protocol: not used here. D50 is a vesicant: patent vein, then flush. D5LR continues during rescue.
GDM/type 2: hold IV insulin during rescue and restart per the nomogram row. Type 1: resume 0.5 units/h once >70 and rising (rising = higher than the previous check).

SICK, KETONES, VOMITING? → DKA · DARK GRAY

Pregnancy DKA can occur below 200 mg/dL; there is no glucose gate for ketone testing when ill. Check beta-hydroxybutyrate, VBG, electrolytes/K+, creatinine and anion gap; maternal assessment and fetal monitoring. Use the CSM DKA order set, not this chart. Reference only, per CSM DKA order set: insulin 0.1 units/kg/h once K+ is safe. Type 1 on plain LR with positive ketones comes here; no ad hoc dextrose. Delivery is not DKA treatment.

A · AFTER DELIVERY (CSM)	Notify the provider when delivery is complete to start the CSM Postpartum Diabetes Management Order Set (doses there). GDM: stop insulin at placenta; 75-g 2-hour OGTT at 4–12 weeks postpartum. Type 1: basal always covered. Continue IV insulin 1–2 hours after the first SC basal dose; then stop D5LR and insulin together.
B · NEWBORN	Tell the nursery the diabetes type, treatment, last maternal glucose and steroids. Stable baby: feed within 1 hour of birth; the nursery team owns the neonatal glucose protocol.
C · STEROIDS / TIMING	Steroids raise glucose up to 5 days: more FSBG checks; follow the nomogram. SMFM #58: no late-preterm (34+0–36+6) steroids in pregestational diabetes. Timing and EFW ≥4,500 g counseling: Diabetes in pregnancy card.
D · NOTIFY	RN to OBHG hospitalist: any FSBG <70 or any rescue; 61–90 on a type 1 floor; 5 units/h with rising values; ketones; any gap in type 1 basal; diet/oral group: >180, ketones or illness.

Doses to be checked against CSM pharmacy build. Decision support only, not orders or unit policy. FSBG = fingerstick blood glucose; BHB = beta-hydroxybutyrate; POC = point of care. **Cite:** CHRISTUS Health OB insulin infusion protocol (CSM, transcribed 11 Nov 2026); ACOG PB 190 and 201; ADA Standards of Care 2026 §6, §15–16; SMFM Consult #58.

Last reviewed: Nov 2026